

AMaC

A. Mansour & Colleagues · Master Series

PLAB 2 REVISION

THE FINAL FORTNIGHT

135 OSCE cards for the last two weeks

UKMLA / PLAB 2 · CPSA-aligned · 70 ★ must-know stations
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AMaC · PLAB 2 Revision · The Final Fortnight

AMaC ? Preview · The Final Fortnight · Not for distribution

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How to Use This Book

This is a crammer for the final two weeks — a distilled, retention-focused companion to the AMaC PLAB 2 Master Series. Every card is one page (a few carry a longer script), built to be scanned, recalled and rehearsed, not read cover to cover.

Mapped to the four PLAB 2 examiner domains — **data gathering, clinical management, safety, and communication**.

Read it the right way: say the lines out loud and rehearse them. **If you are reading this passively, you are using it incorrectly.**

The four parts — and what the examiner is really asking

Part A — The Exam Engine. *“Can you think safely?”*

Part B — Acute & Emergency Stations. *“Can you recognise danger fast?”*

Part C — Specialty & Communication. *“Can you communicate like a clinician?”*

Part D — Examinations, Procedures & Interpretation. *“Can you perform?”*

How each card is built

Banner — the station, with ★ **MUST-KNOW** flag and frequency/risk tags.

Blueprint — the emergency flow or consultation spine, at a glance.

Key Frameworks — the mnemonics, algorithms and the few numbers that matter.

Gold-Standard Script — (Part C) the exact words to say, for the communication stations.

Instant Fail — what loses the station, paired with the line that saves it.

Must-Say Lines, Key Rule, Common Mistakes, Curveball, Gold Closing — the marks you must bank, and the summary to end on.

Every section exists because it scores marks — nothing here is padding. The blueprint banks structure marks, the frameworks the knowledge marks, the instant-fail and must-say lines the safety and communication marks, and the closing the global rating.

Use the scripts as scaffolding, not a recitation — learn the shape, then sound like yourself. Examiners can hear memorisation.

How to read the markers

★ **MUST-KNOW** — a core-exam-path card; drill these first.

HIGH / VERY HIGH — how often the station comes up.

RED PATH — time-critical: minutes matter, escalate.

INSTANT-FAIL RISK — a single slip fails the whole station.

The ★ MUST-KNOW system

The 70 starred cards are the core exam path — if time is short, drill these cold first. The Core Exam Path index on the next pages lists them all.

Time-triage: under 7 days to go — work the ★ cards only. Under 3 days — run the ★ cards twice daily: morning recall, evening rehearsal.

A note on currency: clinical thresholds were reviewed against UK guidance current to June 2026 (NICE/CKS, Resus Council UK, BNF). The content is a revision aid and does not replace your own clinical judgement or up-to-date guidelines.

The 14-Day Plan

Front-load the foundations, then layer on the volume. Week 1 builds the engine and the practical skills; Week 2 drills the acute and communication stations. Day 14 is a pure ★ review.

DAY	FOCUS	WHAT TO DRILL
1–2	Part A · Exam Engine	The universal blueprint, ABCDE, consent, breaking bad news, safety-netting, SBAR, reasoning aloud.
3–4	Part D · Examinations	The five system exams + thyroid, breast, MSE — rehearse the verbalisation out loud.
5–6	Part D · Procedures & Interpretation	Cannulation, catheter, suturing, airway; ECG, ABG, CXR, bloods; killer-drug prescribing.
7	Week 1 review	Re-drill every Part A & D ★ card; rehearse openings and closings until automatic.
8–9	Part B · Acute (1)	Resus, chest, cardio, sepsis, neuro, metabolic — the golden-minute flows.
10–11	Part B · Acute (2)	Abdo/surgical, specialty, haem-onc, O&G, paediatrics — recognise, act, escalate.
12	Part C · Communication	Chronic reviews + the script-box stations: rehearse the Gold-Standard Scripts aloud.
13	Part C · Mental health & safeguarding	Risk assessment, DNACPR, palliative, safeguarding — the hardest conversations.
14	Final ★ review	All 70 must-know cards, fast. Sleep well. You are ready.

Three rules for every day:

1. Two passes a day — morning recall, evening rehearsal.

2. Say every answer out loud — silent review does not translate into marks.

3. From Day 5, one timed partner simulation a day — rehearse under pressure, not just on paper.

Confidence self-tracker

Rate each block before and after your revision. Be honest — your reds are your revision plan. Shade or tick: ● not yet ● getting there ● exam-ready.

REVISION BLOCK	CONFIDENCE (4 passes)
Part A · Exam Engine	○ ○ ○ ○
Part B · Resus, Chest & Cardio	○ ○ ○ ○
Part B · Neuro, Metabolic & Abdo	○ ○ ○ ○
Part B · Specialty, Haem-Onc, O&G & Paeds	○ ○ ○ ○
Part C · Chronic Reviews & Women's Health	○ ○ ○ ○
Part C · Minor Illness, Older & Palliative	○ ○ ○ ○
Part C · Mental Health & Safeguarding	○ ○ ○ ○
Part D · Examinations	○ ○ ○ ○
Part D · Procedures, Prescribing & Interpretation	○ ○ ○ ○

Any red block becomes tomorrow's first job — don't progress past it until it's at least amber.

The Core Exam Path

If time is short, this is your fast track: the 70 ★ MUST-KNOW cards. Drill every one of these cold before you touch the rest.

Part A · The Exam Engine 8 of 11 cards

A1 The Universal Station Blueprint **A2** ABCDE & the Golden Minute **A3** Consent, Capacity & Best Interests **A4** Breaking Bad News & Difficult Conversations **A5** Red Flags, Safety-Netting & Risk Communication **A6** SBAR & Safe Escalation **A10** Visible Clinical Reasoning & Safe Verbalisation **A11** Prescribing & Calculation

Part B · Acute & Emergency Stations 40 of 65 cards

B1 Cardiac Arrest & Peri-Arrest Rhythms **B3** Anaphylaxis **B4** Acute Chest Pain & ACS
B5 Acute Dyspnoea — the Breathless Patient **B6** Acute Asthma **B7** COPD Exacerbation
B8 Community-Acquired Pneumonia **B9** Pulmonary Embolism **B10** Pneumothorax (incl. tension) **B12** Acute Heart Failure / Pulmonary Oedema **B13** Acute Arrhythmias (brady & tachy) **B15** Sepsis & Septic Shock **B16** Acute Stroke & TIA
B17 Reduced Consciousness / the Unresponsive Patient **B18** Seizures & Status Epilepticus
B19 Acute Headache & Subarachnoid Haemorrhage **B20** Bacterial Meningitis **B22** Acute Back Pain & Cauda Equina **B23** Head Injury **B24** Diabetic Ketoacidosis (DKA) **B26** Hypoglycaemia **B27** Acute Kidney Injury **B30** Toxicology & Poisoning
B31 Acute Behavioural Disturbance **B32** Acute Abdomen **B33** Upper GI Bleed **B34** Acute Pancreatitis **B37** The Surgical Referral (?appendicitis / ?cholecystitis)
B38 Sudden Visual Loss **B40** Upper Airway Emergencies & Stridor **B41** Testicular Torsion
B45 Neutropenic Sepsis **B49** Metastatic Spinal Cord Compression **B50** Septic Arthritis **B51** Compartment Syndrome **B53** Ectopic Pregnancy **B55** Pre-eclampsia & Eclampsia **B56** Postpartum Haemorrhage **B60** The Febrile Child & Paediatric Sepsis **B61** Paediatric Respiratory Emergencies

Part C · Specialty & Communication 14 of 40 cards

C1 Cardiovascular Risk & Diabetes Annual Review **C10** Sexual Health & STIs **C24** DNACPR & Escalation Conversations **C25** Palliative & End-of-Life Care **C26** Depression & Anxiety **C29** Self-harm & Suicide Risk Assessment **C30** Alcohol Misuse
C31 Eating Disorders — Acute Medical Risk **C33** Chronic Pain & Persistent Physical Symptoms **C34** Sexual Assault & FGM **C35** Child Safeguarding & Non-Accidental Injury
C38 Pre-operative Assessment **C39** The Angry or Upset Patient **C40** Duty of Candour & Open Disclosure

Part D · Examinations, Procedures & Interpretation 8 of 19 cards

D1 The Examination Foundation **D2** Cardiovascular Examination **D3** Respiratory Examination **D4** Abdominal Examination **D5** Neurological & Cranial Nerve Examination
D6 Musculoskeletal & GALS Examination **D15** Killer Drugs & Fluid Prescribing **D16** ECG Interpretation

Master these 70 and you are safe in almost any station.

If You Learn Nothing Else — the Top 25

Inside the 70 is a harder core. Learn only these and you are still dangerous in the exam — the universal skills, the killer emergencies, the feared conversations, and the must-read investigations.

1	A1	The Universal Station Blueprint
2	A2	ABCDE & the Golden Minute
3	A3	Consent, Capacity & Best Interests
4	A4	Breaking Bad News & Difficult Conversations
5	A5	Red Flags, Safety-Netting & Risk Communication
6	A10	Visible Clinical Reasoning & Safe Verbalisation
7	A11	Prescribing & Calculation
8	B1	Cardiac Arrest & Peri-Arrest Rhythms
9	B3	Anaphylaxis
10	B4	Acute Chest Pain & ACS
11	B5	Acute Dyspnoea — the Breathless Patient
12	B15	Sepsis & Septic Shock
13	B16	Acute Stroke & TIA
14	B24	Diabetic Ketoacidosis (DKA)
15	B45	Neutropenic Sepsis
16	B60	The Febrile Child & Paediatric Sepsis
17	C1	Cardiovascular Risk & Diabetes Annual Review
18	C24	DNACPR & Escalation Conversations
19	C25	Palliative & End-of-Life Care
20	C26	Depression & Anxiety
21	C29	Self-harm & Suicide Risk Assessment
22	C34	Sexual Assault & FGM
23	D1	The Examination Foundation
24	D15	Killer Drugs & Fluid Prescribing
25	D16	ECG Interpretation

Morning of the Exam — the Non-Negotiables

Don't revise new content today. Read this once, breathe, and walk in trusting your preparation. In every station, these are the marks you must not leave on the table:

Open every station — Wash hands · introduce yourself · confirm name & DOB · gain consent.

Explore the person — Ideas, Concerns, Expectations — early, and weave them into the plan.

Stay safe first — Treat as you go in ABCDE · act before you talk · escalate early with SBAR.

Screen the red flags — Ask them, name them, and safety-net: what to watch for, when and where to act.

Before any drug — Check allergies · the dose, decimal-safe · 'units' in full.

For intimate exams — Offer and document a chaperone — every time.

Think out loud — Say what you see, what it means, and what you'll do — visible reasoning scores.

Close kindly — Summarise · check understanding · safety-net · thank the patient.

You are ready. Be structured, be safe, be kind.

PART

A

The Exam Engine

The universal skills every station is built on — structure, safety, reasoning and communication.

11 cards · 8 ★ must-know

★ **MUST-KNOW A1 · The Universal Station Blueprint**

CORE · ALL PATHWAYS · the spine of every encounter

BLUEPRINT

INTRODUCE → OPEN → HISTORY (+ red flags) → ICE → SUMMARISE → EXPLAIN → PLAN → SAFETY-NET → CHECK → CLOSE

KEY FRAMEWORKS

ICE — Ideas · Concerns · Expectations — ask early, validate, weave into the plan
Safety-net (4W) — What to watch for · When to act · Where to go · if Worse
Managing uncertainty — “At this stage...” · “Based on what we know so far...”

INSTANT FAIL — WHAT LOSES THE STATION, AND THE LINE THAT SAVES IT

✗ LOSES THE STATION	✓ THE LINE THAT SAVES IT
Rambling / no structure	“Let me structure this — your story, your ideas and concerns, then a plan.”
ICE not explored	“What do you think might be going on, and what worries you most?”
Anchoring / premature closure	“What else could this be? Let me check the red flags.”
No safety-net	“If X happens, go to Y within Z.”
Silent reasoning	“Let me talk you through my thinking...”

MUST-SAY SCORING LINES

“What do you think is happening?” · “What worries you most?” · “What were you hoping I could do?” · “Just to check I’ve understood...” · “Does that make sense? Anything else you’d like to cover?”

KEY RULE

Timing: intro by 0:20 · ICE + red flags by 2:30 · reasoning 2:30–4:30 · management 4:30–6:30 · close by 8:00.

COMMON MISTAKES

Skipping ICE · anchoring on the first diagnosis · stopping the history too early · running out of time · explaining in jargon.

GOLD CLOSING STATEMENT

“I structured the consultation, explored the patient’s ideas, concerns and expectations, shared the plan, and safety-netted clearly.”

★ **MUST-KNOW A2 · ABCDE & the Golden Minute**

CORE · RED PATH · INSTANT-FAIL RISK

BLUEPRINT

Golden Minute (Safety → Responsiveness → Call help → Start ABCDE → Attach monitoring) → A·B·C·D·E, treat as you go → Reassess after every step → Escalate early → Transition

KEY FRAMEWORKS

A — Airway — stridor/gurgling → adjuncts, suction, call anaesthetics
B — Breathing — O₂ only if hypoxic (94–98%, 88–92% if COPD risk); treat PTX/asthma/PE
C — Circulation — IV access · bloods · fluids if shocked · reassess response
D — Disability — glucose first · GCS/AVPU · pupils
E — Exposure — temperature · rash · bleeding · dignity

INSTANT FAIL — WHAT LOSES THE STATION, AND THE LINE THAT SAVES IT

✗ LOSES THE STATION	✓ THE LINE THAT SAVES IT
Full history before stabilising	"I'll stabilise with ABCDE first; history once stable."
Silent assessment	"I'll verbalise each step as I go."
Ignoring abnormal observations	Act on them immediately and say so
No reassessment after a step	"I'll reassess after every intervention."
No escalation	"I'm calling for senior help now."

MUST-SAY SCORING LINES

"Treat as I go, not assess then treat." · "15 L/min via non-rebreathe, then titrate to target." · "If the patient deteriorates I return to A." · "Reassess and verbalise the response after every intervention."

KEY RULE

Verbalisation loop, every step: what I'm assessing · what I'm worried about · what I'm doing about it.

COMMON MISTAKES

History before stability · silence · forgetting glucose in D · moving past an unstable step · late escalation.

CURVEBALL

Asked to hand over mid-resuscitation → give a 30-second SBAR without abandoning reassessment, and name who continues A–E.

GOLD CLOSING STATEMENT

"I used the Golden Minute to act on immediate threats, completed ABCDE systematically, treated as I went, reassessed after each step, and escalated early."

★ **MUST-KNOW A3 · Consent, Capacity & Best Interests**

VERY HIGH · RED PATH · INSTANT-FAIL RISK

BLUEPRINT**Check capacity (URWC) → Benefits → Risks (CMSM) → Alternatives (incl. no treatment) → Teach-back → Confirm voluntary → Document (DISCUSS)****KEY FRAMEWORKS****MCA 2-stage test** — 1) impairment of mind/brain? 2) can they Understand · Retain · Weigh · Communicate?**5 MCA principles** — presume capacity · support · unwise ≠ incapable · best interests · least restrictive**Risks (CMSM)** — Common · Serious · Material (patient-specific, Montgomery) · Mortality**Lacks capacity** — best interests via SILVER; only Health & Welfare LPA can decide**Under-16s — Gillick vs Fraser** — Gillick competence = a child under 16 with the maturity to understand and consent to treatment in general · Fraser guidelines = the specific criteria for giving contraception or sexual-health advice without parental knowledge**INSTANT FAIL — WHAT LOSES THE STATION, AND THE LINE THAT SAVES IT**

✗ LOSES THE STATION	✓ THE LINE THAT SAVES IT
Proceeding without checking capacity	"First, I'll check you're able to make this decision today."
Overriding a capacitous refusal	"You have capacity, so I respect your decision."
Letting family decide for a capacitous adult	"The decision rests with the patient."
No risk discussion before a procedure	Give common, serious and material risks (CMSM)
Writing only "consent obtained"	Document benefits, risks, alternatives and teach-back

MUST-SAY SCORING LINES

"Capacity is decision- and time-specific — I assume it but check it." · "An unwise decision doesn't mean a person lacks capacity." · "The risks that matter are the ones important to you (Montgomery)." · "Alternatives include doing nothing." · "In your own words, what do you understand?" · "For under-16s: Gillick competence for treatment, Fraser criteria for contraception."

KEY RULE

A capacitous adult (including a pregnant woman) may refuse any treatment.

Children: Gillick for any treatment under 16; Fraser for contraception only (all five criteria).

COMMON MISTAKES

Assuming capacity · jargon · no teach-back · missing the material (patient-specific) risk · accepting a family veto.

GOLD CLOSING STATEMENT

"I obtained valid consent — capacity assessed on the two-stage test, benefits and CMSM risks explained, alternatives including no treatment discussed, understanding confirmed, and the decision documented."

★ MUST-KNOW A4 · Breaking Bad News & Difficult Conversations

CORE · ALL PATHWAYS · INSTANT-FAIL RISK

BLUEPRINT

SPIKES — Setting → Perception → Invitation → Knowledge (warning shot) → Emotion (NURSE) → Strategy & summary

KEY FRAMEWORKS

NURSE — Name · Understand · Respect · Support · Explore the emotion

Difficult conversation — Acknowledge → Explore → Agree a plan

Duty of Candour — Inform · Apologise (unconditionally) · Explain · Support · Document · escalate + Datix

INSTANT FAIL — WHAT LOSES THE STATION, AND THE LINE THAT SAVES IT

✗ LOSES THE STATION	✓ THE LINE THAT SAVES IT
Bad news with no warning shot	"I'm afraid I have some difficult news to share..."
Conditional apology	"I'm sorry this happened." (never "sorry you feel...")
Ignoring emotion	Use NURSE — name and acknowledge it
"I understand how you feel."	"I can't imagine exactly how this feels."
Detailed prognosis unasked	Chunk and check first; follow the patient's cues

MUST-SAY SCORING LINES

Warning shot · then silence · "This is a huge shock." · "I'm here with you." · "What concerns you most right now?" · "Does that make sense so far?"

KEY RULE

Fire a warning shot, then pause and let silence do the work before any detail.

COMMON MISTAKES

No warning shot · talking over silence · false empathy · info-dumping prognosis · a conditional apology.

GOLD CLOSING STATEMENT

"I warned before telling, delivered the news clearly and compassionately, responded to emotion, checked understanding, and agreed follow-up."

★ MUST-KNOW A5 · Red Flags, Safety-Netting & Risk Communication

CORE · ALL PATHWAYS · INSTANT-FAIL RISK

BLUEPRINT

RECOGNISE (red flags) → ACT (manage / escalate) → COMMUNICATE (safety-net + absolute risk)

KEY FRAMEWORKS

Safety-net — What to watch for · Where to go · When to act (+ dropped-call plan if remote)

Risk — absolute numbers / natural frequency — “out of 100 people, about X...”, never relative %

High-yield red flags — headache: thunderclap/neck stiffness · back: saddle numbness/retention · child fever: non-blanching rash

INSTANT FAIL — WHAT LOSES THE STATION, AND THE LINE THAT SAVES IT

✗ LOSES THE STATION	✓ THE LINE THAT SAVES IT
Missing a red flag in the brief	“I’ll screen for serious causes before we go further.”
Vague safety-netting	“If X happens, go to Y within Z.”
Relative risk only	Convert to absolute: “from 2 in 100 to 1 in 100.”
No escalation when indicated	“I’m concerned — this needs senior review.”
Over-reassurance	Use conditional, safety-aware language

MUST-SAY SCORING LINES

“I need to check for some serious causes — is that okay?” · “Out of 100 people, about X...” · “If anything worsens, seek urgent help.” · “I’ll document the safety-netting.”

KEY RULE

A red flag stated in the brief or by the patient and not acted on is an instant-fail risk.

COMMON MISTAKES

Missed red flag · vague advice · relative risk · silent reasoning · over-reassurance.

GOLD CLOSING STATEMENT

“I screened for red flags, communicated risk in absolute terms, gave clear safety-netting, and escalated appropriately.”

★ **MUST-KNOW A6 · SBAR & Safe Escalation**

CORE · RED PATH · the pass/fail moment

BLUEPRINT

SBAR — Situation (who · where · the problem) → Background (history · meds · obs) → Assessment (findings · concern · differential) → Recommendation (what you need — always)

KEY FRAMEWORKS

Escalate when — unstable · red flags · diagnostic uncertainty · safeguarding · beyond your competence

INSTANT FAIL — WHAT LOSES THE STATION, AND THE LINE THAT SAVES IT

✗ LOSES THE STATION	✓ THE LINE THAT SAVES IT
Not escalating when indicated	"I'll escalate to a senior to ensure safe care."
Handover with no recommendation	State exactly what you need done
Vague, rambling handover	Use the four structured SBAR sections
Not naming who you're calling	"I'll call the medical registrar now."

MUST-SAY SCORING LINES

"I'm concerned about this patient and I need..." · name the person/role · "Please could you review within [time]?" · close the loop: confirm what will happen next.

KEY RULE

Failure to escalate when indicated is an instant fail — the Recommendation is the part candidates omit.

COMMON MISTAKES

No recommendation · no named recipient · escalating too late · an unstructured handover.

GOLD CLOSING STATEMENT

"I recognised the patient needed senior input, escalated promptly using SBAR, and made a clear, specific recommendation."

A7 · Safeguarding, Interpreters & Cultural Competence

VERY HIGH · safety + legal · high fail-risk

BLUEPRINT

Safeguarding — RECOGNISE → RESPOND → REFER → RECORD

KEY FRAMEWORKS

Absolute child triggers — under-13 sexual activity · disclosure of abuse · injury in a non-mobile child · patterned bruising · STI/pregnancy under 16

Adults with capacity — may decline referral unless serious risk to others, coercion, serious crime, or public interest

Mandatory — FGM in anyone under 18 → report to police

INSTANT FAIL — WHAT LOSES THE STATION, AND THE LINE THAT SAVES IT

✗ LOSES THE STATION	✓ THE LINE THAT SAVES IT
Promising confidentiality in safeguarding	"I can't promise to keep this secret if you or others are at risk."
Using a child/family member to interpret	"I'll arrange a professional interpreter."
Not seeing a vulnerable patient alone	See them alone before exploring concerns
Missing a mandatory FGM report (under 18)	Report to police; complete safeguarding referral

MUST-SAY SCORING LINES

"Are there cultural or religious beliefs we should consider?" · "I'll arrange a professional interpreter." · use the patient's affirmed name and pronouns · "Is anyone making you feel unsafe?"

KEY RULE

Interpreters: professional only — never a child, and avoid family for sensitive topics; speak to the patient, not the interpreter.

COMMON MISTAKES

False promise of confidentiality · family interpreter · not seeing the patient alone · missing a mandatory report.

GOLD CLOSING STATEMENT

"I recognised the safeguarding concern, saw the patient alone with a professional interpreter, referred appropriately, and documented clearly."

A8 · Remote & Telephone Consultations

HIGH · INSTANT-FAIL RISK

BLUEPRINT

Opening Golden Minute: ID → location → privacy (“are you alone and able to speak freely?”) → connection / dropped-call plan → consent → then **SAFE-CALL**

KEY FRAMEWORKS

SAFE-CALL — Situation · Assessment (probe + red flags) · Findings · Explanation (limits) · Contingency · Advice (+ safety-net) · Loop (consent · follow-up · GP)

Exam proxies — breathless? → “full sentences?” · rash → “press a glass — does it blanch?” · pain → “can you walk?”

INSTANT FAIL — WHAT LOSES THE STATION, AND THE LINE THAT SAVES IT

✗ LOSES THE STATION	✓ THE LINE THAT SAVES IT
No ID / location / privacy / consent / dropped-call check	Do all five at the very start
Keeping an unsafe patient on the phone	“Based on [red flag], you need a face-to-face review.”
Remote prescribing of controlled / high-risk drugs	Avoid; defer to face-to-face
Vague safety-net / no dropped-call plan	Use the four-part remote safety-net

MUST-SAY SCORING LINES

the five opening checks · “I can take a history and advise, but if I’m concerned you’ll need to be seen.” · “If we’re disconnected I’ll call back; if you can’t reach me, call 999.”

KEY RULE

Remote = more risk → more structure → more safety-netting → more documentation.

COMMON MISTAKES

Skipping the opening checks · no exam proxy · managing an unsafe patient remotely · no dropped-call plan.

GOLD CLOSING STATEMENT

“I confirmed identity, location, privacy and consent, used exam proxies, recognised the limits of remote care, and safety-netted including a dropped-call plan.”

A9 · Exam-Day Technique & Recovery

CORE · performance, not perfection

BLUEPRINT

The 8-minute station: 0:00–0:20 intro/verify/agenda → 0:20–2:30 open + ICE + red flags → 2:30–4:30 summarise + think aloud → 4:30–6:30 explain + plan + safety-net → 6:30–8:00 check + close

KEY FRAMEWORKS

Read the brief — TASK (what am I doing?) · PERSON (who's in the room?) · TWIST (what's the challenge?)

Kill words — “angry” → de-escalate · “relative” → confidentiality · “refuses” → capacity · “sudden” → red flag · “distressed” → acknowledge emotion

Reset — 3-breath reset before each station · STOP-RESET-GO after a bad one

INSTANT FAIL — WHAT LOSES THE STATION, AND THE LINE THAT SAVES IT

✗ LOSES THE STATION	✓ THE LINE THAT SAVES IT
Rushed communication	Slow down — mark each word; lower the pitch
Loss of structure under nerves	Fall back on the 8-minute map
Visible panic	3-breath reset; “Safety first.”
Silence or rambling	Think aloud, but signpost and stay structured

MUST-SAY SCORING LINES

intro in the first 10 seconds · set an agenda · think aloud · safety-net before closing.

KEY RULE

After a bad station: “That station is over — I can’t change it.” Reset and walk in as if it were station 1.

COMMON MISTAKES

Re-living a bad station · dropping structure under pressure · complacency by station 18 · rushing the close.

GOLD CLOSING STATEMENT

“I read the brief for task, person and twist, held my structure under pressure, reset between stations, and performed station 18 like station 1.”

★ MUST-KNOW A10 · Visible Clinical Reasoning & Safe Verbalisation

CORE · HIGH YIELD · silence = no marks

BLUEPRINT

GATHER → ORGANISE → PRIORITISE → JUSTIFY → SYNTHESISE → PLAN → SAFETY-NET. Golden 30 seconds (by minute 3–4): “Let me organise my thoughts. Most likely is X because Y. I must not miss Z.”

KEY FRAMEWORKS

Five core steps — Most likely (because) · Must-not-miss · Justify tests (BECAUSE rule) · SIPS synthesis · Escalate + safety-net

3-tier differential — most likely · life-threatening · other possibilities

Safe verbalisation (the rules) — reason aloud TO THE EXAMINER, not alarmingly AT the patient · don't voice an unconfirmed serious diagnosis to the patient · signpost ('let me think this through') · keep patient-facing language plain and reassuring

INSTANT FAIL — WHAT LOSES THE STATION, AND THE LINE THAT SAVES IT

✗ LOSES THE STATION	✓ THE LINE THAT SAVES IT
Unsignposted silence over 10 seconds	“Let me organise my thoughts...”
Ordering a test with no reason	BECAUSE rule: “...because it would change management.”
No life-threatening differential	“I must not miss [X] because...”
Voicing a scary differential at the patient	Reason for the examiner; keep patient-facing words measured
Anchoring / premature closure	State alternatives; update the differential aloud

MUST-SAY SCORING LINES

“Most likely is X because Y.” · “I must not miss Z.” · “I'd request [test] because it would change management.” · “Putting this together, the pattern fits...” · “This new finding changes my thinking...”

KEY RULE

Examiners score visible reasoning — say what you see, what it means, and what you'll do.

COMMON MISTAKES

Silent excellence (reads as absence) · tests without justification · no must-not-miss · failing to update aloud.

CURVEBALL

You realise mid-station your diagnosis was wrong → say so: “I'm revising my impression because of X.” The correction scores more than the original.

GOLD CLOSING STATEMENT

“Most likely is X because Y; I must not miss Z; I'd request [test] because it would change management — putting it together, the pattern fits X, and I'll safety-net and escalate if needed.”

★ **MUST-KNOW A11 · Prescribing & Calculation**

CORE · numeracy · INSTANT-FAIL RISK

BLUEPRINT

Read the order → identify what to calculate (dose by weight, infusion rate, concentration, conversion) → calculate → double-check (units, decimal, order of magnitude) → state the working aloud → sanity-check against the patient

KEY FRAMEWORKS

Core calculations — dose by weight ($\text{mg/kg} \times \text{kg}$) · infusion rate ($\text{mL/hr} = \text{volume} \div \text{hours}$) · concentration (mg/mL) · unit conversion ($\text{mg} \leftrightarrow \text{mcg} \leftrightarrow \text{g}$, always $\times/\div 1000$)

Decimal & unit safety — leading zero, no trailing zero (0.5 mg, not .5 mg or 5.0 mg) · ‘units’ in full for insulin · double-check the order of magnitude

Worked example — 1 L over 8 h = $1000 \div 8 = 125 \text{ mL/hr}$ · a drug at X mg in Y mL → mg/mL, then the volume needed for the dose

Always — show the working, state the units at every step, and sanity-check — ‘does this dose or volume make sense for this patient?’

INSTANT FAIL — WHAT LOSES THE STATION, AND THE LINE THAT SAVES IT

✗ LOSES THE STATION	✓ THE LINE THAT SAVES IT
A decimal / order-of-magnitude error	Double-check; leading zero, no trailing zero
A wrong unit conversion (mg / mcg)	$\times 1000$ carefully, units at every step
Not showing the working	State each step

MUST-SAY SCORING LINES

“I’ll show my working with units at each step.” · “Leading zero, no trailing zero.” · “Let me double-check the order of magnitude.” · “Does this dose make sense for this patient?”

KEY RULE

An unchecked decimal point is a tenfold error — re-read the number and sanity-check the answer against the patient.

COMMON MISTAKES

A decimal error · a wrong conversion · not showing the working · no sanity check.

GOLD CLOSING STATEMENT

“I identified the calculation needed, worked it through with units at each step, double-checked the decimal and order of magnitude, and sanity-checked the answer against the patient before prescribing.”

PART

B

Acute & Emergency Stations

*The time-critical presentations. Recognise the danger,
act in the golden minute, escalate.*

65 cards · 40 ★ must-know

★ **MUST-KNOW B1 · Cardiac Arrest & Peri-Arrest Rhythms**

HIGH · ALL PATHWAYS · INSTANT-FAIL RISK

BLUEPRINT

Confirm arrest (no signs of life, pulse check ≤10 s) → CPR 30:2 → attach defibrillator → rhythm check every 2 min → shockable vs non-shockable branch → adrenaline ± amiodarone → treat 4 Hs / 4 Ts → post-ROSC care

KEY FRAMEWORKS

CPR quality — 100–120/min · depth 5–6 cm · full recoil · 30:2 · swap compressor every 2 min · pulse check ≤10 s

Shockable (VF / pVT) — shock 150–200 J → 2 min CPR → adrenaline 1 mg + amiodarone 300 mg after the 3rd shock (150 mg for 2nd)

Non-shockable (PEA / asystole) — adrenaline 1 mg IV now, then every 3–5 min; do not shock

4 Hs / 4 Ts — Hypoxia · Hypovolaemia · Hypo/Hyper-K⁺ + acidosis · Hypothermia // Tension PTX · Tamponade · Toxins · Thrombosis (PE/ACS)

Peri-arrest rule — Ask first: unstable? (shock · syncope · ischaemia · heart failure) — unstable tachy → synchronised cardioversion; unstable brady → atropine 500 mcg (to 3 mg) then pacing

Post-ROSC — O₂ 94–98% (avoid hyperoxia) · 12-lead ECG ≤10 min (STEMI → cath lab) · treat the cause · prevent fever · MAP >65

INSTANT FAIL — WHAT LOSES THE STATION, AND THE LINE THAT SAVES IT

✗ LOSES THE STATION	✓ THE LINE THAT SAVES IT
Pulse check over 10 s / delaying CPR	"No definite pulse in 10 seconds — starting CPR."
Poor-quality CPR	"100–120/min, 5–6 cm, full recoil, swap every 2 minutes."
Misidentifying the rhythm	"Stop compressions — rhythm check: is this shockable?"
Adrenaline at the wrong time	"Shockable → after the 3rd shock; non-shockable → now."
Wrong peri-arrest branch	"First — is the patient stable or unstable?"

MUST-SAY SCORING LINES

"No signs of life — high-quality CPR, 30:2." · "Adrenaline 1 mg after the 3rd shock, then every 3–5 min." · "Amiodarone 300 mg after the 3rd shock." · "Running the 4 Hs and 4 Ts." · "Post-ROSC: titrate oxygen, 12-lead ECG, treat the cause."

KEY RULE

Lead out loud — assign roles by name: "You, compressions; you, airway; you, IV access and drugs."

COMMON MISTAKES

Poor-quality CPR · wrong drug timing · missing a reversible cause · wrong peri-arrest branch · leaving the patient on 100% O₂ after ROSC.

CURVEBALL

ROSC then re-arrest → restart ALS from the current rhythm and re-run the 4 Hs / 4 Ts to find the cause.

GOLD CLOSING STATEMENT

“I confirmed arrest, delivered high-quality CPR with minimal interruptions, defibrillated shockable rhythms, gave drugs at the correct points, worked through the 4 Hs and 4 Ts, and managed post-ROSC care with oxygen titration and an early ECG.”

B2 · Neonatal Resuscitation

the first 10 minutes · INSTANT-FAIL RISK

BLUEPRINT

Warm / dry / stimulate (+ hat; plastic wrap if <32 weeks) → 30-sec assessment (term? tone? breathing?) → HR is the driver → HR <100: 5 inflation breaths then PPV → reassess at 30 s → HR <60 after effective PPV: 3:1 compressions → adrenaline → escalate

KEY FRAMEWORKS

Thermal care (0–30 s) — warm · dry · remove wet towels · hat · neutral airway; plastic wrap if under 32 weeks

Heart rate drives everything — ≥100 observe · 60–100 start PPV · <60 after effective PPV → compressions

PPV — 5 inflation breaths (2–3 s; 30 cmH₂O term / 25 preterm), then 30/min — confirm chest rise

No chest rise → MR SOPA — Mask seal · Reposition airway · Suction · Open mouth · Pressure ↑ · Alternative airway

Compressions — only if HR <60 after effective PPV · 3:1 · two-thumb · lower third of sternum

Meconium — vigorous baby → do NOT suction; non-vigorous → PPV immediately

INSTANT FAIL — WHAT LOSES THE STATION, AND THE LINE THAT SAVES IT

✗ LOSES THE STATION	✓ THE LINE THAT SAVES IT
HR under 100 and no PPV	"Heart rate under 100 — starting positive-pressure ventilation."
Compressions before 30 s effective PPV	"Ventilation first; compressions only if HR <60 after effective PPV."
Suctioning a vigorous meconium baby	Do not suction — ventilate
Forgetting thermal care	"Warm, dry, hat; plastic wrap if under 32 weeks."
Scoring APGAR before resuscitating	Resuscitate first; score at 1 and 5 minutes

MUST-SAY SCORING LINES

"Heart rate is the decision driver — I reassess every 30 seconds." · "Five inflation breaths, confirming chest rise." · "No chest rise — MR SOPA." · "Compressions 3:1, only after effective ventilation."

KEY RULE

Adrenaline 10–30 mcg/kg IV (1:10,000) only if HR remains <60 despite effective PPV and compressions.

COMMON MISTAKES

Compressions before effective ventilation · wrong ratio · suctioning a vigorous meconium baby · forgetting warmth · silent performance.

CURVEBALL

No chest rise despite PPV → work through MR SOPA before escalating to compressions.

GOLD CLOSING STATEMENT

“I prioritised ventilation guided by heart rate, ensured immediate thermal protection, started PPV early when the heart rate fell below 100, confirmed chest rise, and escalated for compressions only after effective ventilation.”

★ **MUST-KNOW B3 · Anaphylaxis**

VERY HIGH · RED PATH · INSTANT-FAIL RISK

BLUEPRINT

Recognise (ABC compromise + skin/mucosal change) → call for help / 2222
 → **lie flat + legs up** → **IM adrenaline 500 mcg now** → **high-flow O₂ + IV fluids**
 → **reassess, repeat adrenaline every 5 min** → **observe (biphasic)** → **discharge plan**

KEY FRAMEWORKS

Recognise — sudden Airway (swelling/stridor) / Breathing (wheeze/hypoxia) / Circulation (hypotension/collapse) compromise + skin or mucosal change

Adrenaline IM (1:1000) — adult / >12 y 500 mcg · 6–12 y 300 mcg · <6 y 150 mcg · anterolateral thigh · repeat every 5 min

Position — lie flat + legs up; sit up only if breathing is difficult; never stand suddenly

Alongside adrenaline — high-flow O₂ · IV crystalloid 500–1000 mL rapidly · nebulised salbutamol if bronchospasm

No longer routine — steroids and antihistamines (RCUK) — adrenaline is the treatment

After stabilising — observe for a biphasic reaction · discharge with two adrenaline auto-injectors + allergy referral

INSTANT FAIL — WHAT LOSES THE STATION, AND THE LINE THAT SAVES IT

✗ LOSES THE STATION	✓ THE LINE THAT SAVES IT
Delaying adrenaline / antihistamine first	"This is anaphylaxis — IM adrenaline 500 mcg to the thigh now."
IV adrenaline bolus	"IM only; IV adrenaline is specialist-supervised."
Keeping a hypotensive patient upright	"Lie flat, legs raised."
Missing airway compromise	"Airway at risk — urgent anaesthetics."
No auto-injectors / no allergy referral	Two auto-injectors + allergy clinic on discharge

MUST-SAY SCORING LINES

"This is anaphylaxis — treating immediately." · "Adrenaline 500 mcg IM, repeat in 5 minutes if needed." · "Lie flat, legs up." · "Observe for a biphasic reaction." · "Discharge with two auto-injectors and allergy follow-up."

KEY RULE

On a beta-blocker and not responding to adrenaline → consider glucagon and get senior help.

COMMON MISTAKES

Antihistamine/steroid first · IV adrenaline bolus · patient left upright · no repeat dose · no auto-injector plan.

CURVEBALL

On a beta-blocker and adrenaline isn't working → give glucagon and escalate urgently.

GOLD CLOSING STATEMENT

“I recognised anaphylaxis early and treated immediately with IM adrenaline, oxygen and IV fluids using an ABCDE approach, repeated adrenaline as needed, observed for a biphasic reaction, and arranged two auto-injectors with allergy follow-up.”